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TRANSLATION ARTIFACT

NHS PRIMARY CARE

A Plain-Language Companion to the Diagnostic and Redesign Artifacts

“NHS primary care is record-rich and continuity-poor. The patient becomes the record. That is the failure.”

For leadership, clinical teams, policymakers, patients, carers, and builders: this document explains the failure and the replacement in plain language.

ARTIFACT FAMILY — RECOMMENDED READING ORDER				
START	03	Translation Artifact	· This document — begin here	· Public
THEN	01	Diagnostic Artifact	· Full evidence and structural proof	· Public
THEN	02a	Exec Summary	· Public replacement logic	· Public
LAST	02	Redesign Artifact	· Full replacement specification	· Restricted

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CLASSIFICATION	Public — plain-language companion to the Diagnostic and Redesign Artifacts
SOURCE BASIS	Derived from the Diagnostic and Redesign Artifacts. Introduces no independent claims.

START HERE — READING ORDER

Artifact family reading order: Start here (Translation Artifact) → Diagnostic Artifact (full evidence) → Redesign Executive Summary (replacement logic) → Full Redesign Artifact (restricted builder specification). Use this document to create shared understanding. Use the Diagnostic for evidence. Use the Redesign for implementation.

Quick Reference

	SUMMARY
The problem	NHS primary care has no structural mechanism to ensure operational memory is present before clinical decisions are made. The patient becomes the record.
The failure	The GP consultation collapses four system functions into one: memory reconstruction, sensing, interpretation, and clinical authority. This is function fusion, not heavy workload.
The fix	A patient-held operational continuity record. History, symptom events, and structured preparation exist before the consultation. The clinician receives the case; they do not reconstruct it.
The 7 moves	1. Build truth kernel → 2. Symptom event layer → 3. Consultation packet → 4. Outcome append → 5. Emergency access → 6. Carer access → 7. NHS recognition pathway
Proof	Same pressure → different response. Patient no longer repeats history. GP begins at clinical judgment. System functions even when the patient cannot tell the story.
Biggest risk	Recapture: the system pulling custody back into institutions, or the app becoming an audit tool. Patient custody and decision-ready retrieval must be preserved.

Plain-language terms used in this document

TERM	PLAIN MEANING
Operational memory	The usable clinical picture that must be present before the next decision is made. Not a record of the past — the context needed right now.
Ghost structure	The hidden human work — patients repeating history, carers keeping diaries, GPs reconstructing cases — that the architecture should be doing but is not.
Collapse node	The point where too many system failures converge in one place at one moment. Here: the GP consultation.
Function fusion	Several structurally different jobs compressed into one role at the same time. The GP sensing, interpreting, deciding, and remembering simultaneously is function fusion.
Continuity record	The patient's portable operational memory: history, medications, symptom events, clinical outcomes, and pending follow-ups — in one place, held by the patient.
Consultation packet	The one-page structured summary generated before every GP encounter. Readable in under 60 seconds. This is what the redesign delivers to the clinician instead of a cold start.
Recapture	The old failure returning inside the new system. When the app becomes a health journal, or the institution takes custody of the record, recapture has occurred.
Append-only	You can only add to the record. Nothing can be deleted or overwritten. Corrections are added alongside the original so the history is always visible.

SECTION 0

WHAT THIS DOCUMENT IS

This Translation Artifact explains the structural diagnosis and the replacement path in clear, plain language. It is not a policy document, a reform proposal, or a product specification. It is a companion to the Diagnostic and Redesign Artifacts: it introduces no new claims and makes no independent evidence assertions. Its job is translation: to make the diagnosis and replacement understandable without requiring the reader to enter the full evidence base or restricted build specification.

What this document does not say

- It does not say GPs are failing — it says the architecture places impossible structural demands on the GP consultation
- It does not say the 10-minute appointment must be abolished — it says the consultation must begin at clinical judgment, not reconstruction
- It does not say all patients must see the same GP — it says operational memory must be present regardless of which clinician they see
- It does not say the app is a diagnostic tool, a medical device, or a clinical decision system
- It does not say the app replaces the official NHS GP record
- It does not say NHS IT cooperation is required before the redesign delivers value
- It does not claim access to unpublished NHS internal data

SECTION 1

THE SYSTEM IN ONE SENTENCE

NHS primary care operates without structural memory at the point of care, forcing the GP consultation to collapse sensing, interpretation, history reconstruction, and clinical authority into a single time-constrained event.

SECTION 2

WHAT THE SYSTEM SAYS VS WHAT IT DOES

The NHS promises continuity of care: joined-up records, informed decisions, and the right information at the right time. In practice, continuity is often absent before the consultation begins.

The patient supplies history. The carer supplies context. The GP reconstructs the case. The system supplies fragments.

A patient arriving after a hospital stay may find the discharge summary has not arrived. The GP has no complete picture. The patient becomes the record.

The system promises memory. The consultation performs reconstruction.

The NHS also treats the patient as an event — an appointment, a referral, a discharge. But the patient is not an event. Take a patient with diabetes who has three GP appointments, one hospital visit, and two prescription changes over six months. The NHS records each event separately. But the patient's condition exists continuously between those events. The current system has no structural way to connect the dots across care settings. That is why

the patient keeps repeating their history: they are the only thing linking the events together. The system does not remember across encounters unless a human carries the memory.

This is not a clinician failure. It is a structural mismatch between promise and architecture.

SECTION 3

WHERE THE BURDEN LANDS

The system only appears to function because humans carry work the architecture does not. In systems terms, this hidden layer of unrecognised, uncompensated labour is called the ghost structure.

- **Patients carry memory:** history, symptoms, medication context, prior events, and the clinical picture the system cannot retrieve.
- **Carers carry continuity:** logs, timelines, medication diaries, and transition details across care settings.
- **GPs carry fused clinical burden:** sensing, interpretation, memory reconstruction, and authority inside the consultation — simultaneously, under time compression.
- **Receptionists and care navigators carry informal routing:** complexity flagging and continuity brokering the system does not formally assign to anyone.

The carer's health notebook is the ghost structure made physical. The notebook that travels between GPs, hospitals, and specialists — holding the medication changes, the discharge dates, the symptom timeline — is not a lifestyle preference. It is the operational memory the architecture has failed to hold. When we say the redesign succeeds when the carer's diary is retired, we mean it: the notebook is the visible proof of what the system is not doing.

These are not support roles. They are the missing system. Remove them, and the continuity gap becomes visible. The redesign succeeds when these carriers are no longer structurally necessary.

SECTION 4

THE COLLAPSE NODE

The GP consultation has become the collapse node: the point where all of the system's structural failures converge in one place, at one moment, under full clinical consequence.

In a short appointment, the clinician must simultaneously: reconstruct history from whatever the patient can remember; sense presenting symptoms against an incomplete record; interpret those signals into a clinical picture; make a consequential clinical decision; and document the encounter for audit.

That is not simply heavy workload. It is function fusion: four distinct system functions compressed into one human node at the exact point of clinical consequence. The system has routed its structural failure directly into the consultation.

FUNCTION	WHERE IT SHOULD BE	WHERE IT ACTUALLY IS
Sensing	Upstream — before the patient	Inside the consultation — the GP

FUNCTION	WHERE IT SHOULD BE	WHERE IT ACTUALLY IS
	arrives	absorbs symptoms as the patient speaks
Interpretation	Bounded by prior context — available before judgment	Fused with authority — the GP interprets and decides simultaneously
Authority	The GP consultation, acting on structured prior state	The GP consultation, acting on whatever was just reconstructed
Memory	The system — present before the encounter	The patient — reconstructed verbally at the start of every appointment

SECTION 5

COST OF CONTINUATION

The current structure compounds cost and risk as demand increases. Clinical time is spent reconstructing what should already exist. Diagnostic delay becomes normal. Repeat appointments increase. Clinical risk rises at every handoff. Workforce strain accelerates burnout and exit.

Scaling access without fixing continuity does not solve the problem. It sends more patients through the same collapse node. Scaling a broken primitive only scales the failure.

Public evidence already shows the scale:

- Clinical negligence costs approximately £2.8 billion annually
- Coroners issued 36 Prevention of Future Deaths warnings in a single year linked to information-sharing failures
- Over 40% of GPs doubt they will still be practising in five years

These figures are drawn from the Diagnostic Artifact and its cited public sources (NHS Resolution CNSGP 2023/24, Coroner PFD reports 2024, RCGP GP Voice Survey 2024). This Translation Artifact introduces no independent quantitative claims.

These figures do not prove that every cost is caused by the continuity failure. They show the scale of the consequence environment in which the same information-sharing and continuity failure pattern repeatedly appears.

SECTION 6

WHY PREVIOUS FIXES HAVE FAILED

The NHS has added more records, more portals, more patient access features, and more digital programmes. Yet the core question remains unanswered at every consultation:

Is usable clinical continuity present before the decision is made?

A record is not memory. Access is not continuity. Documentation is not retrieval.

The NHS has built audit trace — a record of what happened, produced for compliance and legal defensibility. What it has not built is operational memory — the clinical picture that must

be present before the next decision is made. Every digital programme has improved the audit layer without improving the operational layer.

The system is record-rich and continuity-poor. As long as memory is not structurally present before the consultation, every new digital layer simply relocates the same failure under a new name.

SECTION 7

THE REDESIGN IN PLAIN LANGUAGE

The redesign separates the four functions. It gives each one the right structural home. It does not ask the GP to do less. It asks the architecture to do more.

Before and after

FUNCTION	BEFORE — CURRENT STATE	AFTER — REDESIGNED STATE
Memory	Patient repeats full history at every appointment; GP personal recall for known patients only	Patient-held continuity record; present before every encounter; accessible to any clinician
Sensing	GP absorbs presenting symptoms while simultaneously reconstructing prior state	App captures symptom events upstream; structured prior context available before the encounter begins
Structured preparation	None — GP opens encounter cold	App generates the Consultation Packet: symptom event, relevant history, medications, red flags, pending follow-ups
Authority	GP simultaneously senses, interprets, reconstructs, and decides under time compression	GP receives structured prior state; exercises bounded clinical judgment; appends outcome

The corrected primitive: Operational continuity must be structurally present before the clinical decision, in patient-held form. Clinical authority remains with the clinician.

Before the consultation, the system must carry: memory (patient history, medication context, prior events, unresolved patterns); structured sensing (symptom events captured upstream); and structured preparation (the Consultation Packet).

Inside the consultation, the clinician retains full authority: clinical judgment, diagnosis, prescribing, referral, and treatment decisions. The GP should not begin by reconstructing the case. The GP should receive the case.

This is not a better notes system. It is a reallocation of system functions.

SECTION 7.1

WHAT THE MINIMUM REDESIGN REQUIRES

The redesign is only real if five things exist simultaneously. A deployment missing any one of them is not a replacement. It is a partial correction inside the old frame — and the old failure will return.

1. **A patient-held operational continuity record** — portable, persistent, and retrievable at any point of care by any clinician. If the record is held by an institution rather than the patient, the redesign has not begun.
2. **A consultation packet readable in under 60 seconds** — generated before every encounter from the patient's continuity record. If the GP still begins by reconstructing history verbally, the packet is not doing its job.
3. **Outcome append after every consequential encounter** — the clinical verdict, prescription, referral, and follow-up are recorded and travel to the next consultation. If outcomes disappear after encounters, continuity is not compounding.
4. **Emergency and carer access** — essential fields accessible without authentication for first responders; delegated access for carers with explicit role tagging. If the system fails in the hardest case, it has not been fully built.
5. **A recognition pathway toward NHS ingestion without custody transfer** — the patient holds custody throughout. NHS integration is Move 7, not Move 1. Formal recognition is a later migration step, not a prerequisite for usefulness.

MINIMUM REPLACEMENT RULE

If any one of these five is missing, the change is not a redesign. It is a partial correction inside the old frame. The old failure will return in a new interface.

SECTION 8

THE REDESIGNED OBJECT

The redesign centres on a patient-held operational continuity record. In the proposed redesign, the first carrier for that record is a continuity app, supported by PDF export, emergency access, and carer access. This is not another static record. It is a preparation and memory system designed to make the patient's prior clinical state available before the consultation begins.

At setup

The patient records core continuity information: identity, GP and hospital contacts, confirmed diagnoses, current medications, known allergies, prior events, emergency contacts, and carer details. Minimum setup takes under 7 minutes. The record becomes more complete over time as further encounters are appended. The patient should spend less time on the app than they currently spend repeating their history at each GP appointment.

When symptoms occur

The patient records symptom events in structured form. The app organises these entries into a timeline and surfaces recent changes, unresolved episodes, pending follow-ups, and known risks already recorded in the patient's own continuity record.

The app does not diagnose, triage, prescribe, refer, or hold clinical authority. It organises information so the clinician can judge. Routing prompts, where used, are bounded navigational aids. They do not detect conditions, rank risks, reassure the patient, or make clinical assessments. They only help the patient choose an appropriate route for seeking care based on information the patient entered.

SAFETY INFORMATION

Important for patients and carers: routing prompts in the app are not medical advice. They are based on information you enter. They do not replace a clinician's judgment. If you are worried about your symptoms, seek medical attention — do not wait for the

app to tell you that you are safe. The app is a continuity tool, not a diagnostic service.

Before the consultation

The app generates a consultation packet: the current symptom event, relevant history, recent clinical outcomes, active medications, pending follow-ups, known risks, and unresolved patterns. The Redesign Artifact calls this the Consultation Packet; this document describes it as decision-ready because that is its purpose — the GP is not looking at a list of symptoms, they are looking at a symptom-to-history bridge that has already organised the prior state.

THE 60-SECOND RULE

The packet must be readable in under 60 seconds. If a clinician who has never seen the patient before cannot begin clinical judgment within 60 seconds of opening the packet, the packet has failed. Sixty seconds is not a speed target — it is a structure test. It marks the boundary between a data dump and a minimum viable clinical picture: the prior state a GP needs to act, not the full record of everything that has ever happened.

After the consultation

The outcome is appended to the patient's continuity record: clinical verdict, prescription, referral, advice, follow-up instruction, or no action. This closes the loop. Continuity compounds over time: each appended outcome makes the next consultation better.

Liability boundary: The clinician does not need to rely solely on the patient's memory. Clinical outcomes appended to the record are recorded by the clinician at the time of the encounter. A GP reading the packet is reading structured prior context — not patient-reported clinical verdicts. The clinician still judges. The packet prepares the judgment.

Offline access: The continuity record and consultation packet must be available locally on the device without an internet connection. A patient in a lead-lined hospital basement, or in a rural area without signal, must still be able to present their record. The physical Continuity Card — credit-card sized, printable, QR code to the emergency view — provides access even if the phone is unavailable, uncharged, or damaged.

The app does not replace the clinician. It removes the reconstruction burden before the clinician enters the decision.

SECTION 8.1

PATIENT-HELD ≠ PATIENT-BURDENED

PATIENT-HELD ≠ PATIENT-BURDENED

Patient-held does not mean patient-burdened. The patient holds custody of the continuity object. The architecture must reduce the labour required to maintain, retrieve, and present continuity at every encounter. If the app increases patient administrative burden while calling it empowerment, the redesign has failed.

In the current system, the patient carries continuity as hidden labour: repeating history, maintaining medication logs, bringing handwritten notes. The redesign replaces that burden with custody: the patient holds the record, but the system does the work of organising,

surfacing, and presenting it. The burden moves from the patient's memory into the architecture.

The three-part allocation: The patient holds custody. The system carries memory and preparation. The clinician carries authority. Custody without burden: the patient holds the record, but the system does the work of organising, surfacing, and presenting it.

SECTION 9

THE MOVE ORDER

The redesign must happen in sequence. Right fixes in the wrong order recreate the failure. Each move must be substantially complete before the next begins.

MOVE	WHAT CHANGES	WHY THIS ORDER
1 — Truth Kernel	Build the patient-held continuity data structure and patient onboarding flow. Minimum setup under 7 minutes; extended fields added progressively.	The foundation. Nothing else can be built on an undefined continuity object.
2 — Symptom Event Layer	Build structured upstream symptom capture	Moves sensing upstream; creates the data the packet draws from
3 — Consultation Packet	Build the one-page packet generation for the consultation (decision-ready in under 60 seconds)	The core mechanism. Directly addresses the first structural break.
4 — Outcome Append	Build post-consultation outcome recording into the continuity record	Closes the loop; makes continuity compound across encounters
5 — Emergency Access	Build lock-screen access to essential fields for first responders, plus the physical Continuity Card	Handles the hardest case: when the patient cannot tell the story
6 — Carer Access	Build delegated access mode with explicit role tagging	Handles digital exclusion and low-literacy patients; names the carer role formally
7 — NHS Recognition	Open the formal pathway for NHS ingestion without custody transfer (One-Way Valve)	Deferred until the app has proven value in lived use; NHS integration is Move 7, not Move 1

The full technical specification for each move, including completion criteria, rollback conditions, and structural validation tests, is in the Redesign Artifact.

SECTION 10

WHAT MUST STOP

Before the redesign can hold, the following must stop. These are not adjustments. They are preconditions.

STOP	WHY
Treating the patient as the default memory system	The patient performing the memory function is the first structural break. Building on this assumption replicates the break in a new interface.

STOP	WHY
Treating GP reconstruction as normal clinical work	Reconstruction is not clinical work. It is structural failure surfacing inside the consultation.
Confusing documentation with operational memory	Documentation records what happened. Operational memory surfaces what must be known before the next decision. They are structurally different.
Measuring success by access or adoption rather than retrieval	If the test of success is how many people downloaded the app or logged in, the operational layer remains unmeasured.
Treating handoff failure as a communication problem	The Health Services Safety Investigations Body confirmed it is a structural failure. More guidance without enforcement has not resolved it.
Adding digital access onto fragmented continuity	Each new layer adds documentation density without correcting retrieval. It relocates the failure, not resolves it.
Treating routing prompts as diagnosis	The app must never become a diagnostic authority. If it starts detecting conditions, ranking risks, or reassuring patients that care is unnecessary, it has crossed into a different product class.
Designing the app for daily engagement	The app exists to preserve and present continuity, not to maximise sessions. If engagement becomes the goal, the app becomes a health journal rather than a continuity system.
Treating PDF export as the product	PDF is a fallback. The redesign is the live continuity object and the encounter preparation mechanism.

SECTION 11

WHAT THE SYSTEM MUST NOW CARRY ITSELF

The redesign succeeds only when burden moves from people into structure.

CURRENT BURDEN	MUST BECOME
Patient repeats full history at every appointment	Structured patient-held continuity record, present before the encounter
Carer maintains separate medication logs and appointment diaries	Delegated continuity access; outcome append automates the post-encounter record
GP reconstructs the case from fragments and patient recall	Decision-ready consultation packet generated before the encounter begins
Receptionist routes without structured context	Structured context and bounded safety-routing prompts from the symptom event layer
Repeat unresolved symptoms visible to no one	Repeat-presentation surfacing in the patient's continuity record and packet
Clinical outcome disappears after the encounter	Outcome appended to the continuity record; appears in every subsequent packet

If the GP is still reconstructing the case, the patient is still the memory system, or the app has become another static record, the redesign has not succeeded.

SECTION 12

WHAT SUCCESS LOOKS LIKE

The redesign changes lived experience before it changes institutional metrics. Here is what success looks like for each audience.

AUDIENCE	BEFORE	AFTER
Patient	Repeats full history at every appointment; brings handwritten notes; cannot assert their own history when the system does not have it	Arrives with continuity record intact; no longer reconstructs history verbally; leaves with a record of what was decided
Carer	Maintains separate medication diary; brings appointment logs; acts as the primary memory bridge across care transitions	App holds the outcome record from each encounter; carer no longer maintains a parallel diary; continuity is no longer the carer's burden to organise
GP	Opens every encounter cold; reconstructs from fragments and patient recall; clinical judgment compressed into whatever time reconstruction leaves	Reads the Consultation Packet in under 60 seconds; appointment begins at clinical judgment; outcome appended after the encounter closes the loop
NHS leadership	Diagnostic delay, negligence exposure, GP exit pressure, and repeat PFD warnings from the same information-sharing failure pattern	Measurable reduction in GP reconstruction time; falling history-repetition rate; ghost structure carriers no longer structurally necessary

SECTION 13

WHAT PROVES THE REDESIGN IS REAL

The test is simple:

PROOF CONDITION

Same pressure → different response. The redesign is real when the same clinical workload and patient complexity produces a different encounter: the patient does not repeat their history, the GP begins with the case already structured, and the system functions even when the patient cannot tell the story.

The redesign is confirmed when all of the following hold:

- Patients no longer routinely repeat their full history at each encounter
- For patients using the redesigned route, GPs begin with structured context rather than reconstruction
- Consultations start with clinical judgment, not history reconstruction
- Repeat unresolved presentations are surfaced in the packet before the encounter begins
- Outcomes persist and compound across encounters
- The system functions even when the patient cannot tell the story — unconscious, acutely distressed, or cognitively impaired
- A clinician who has never seen the patient before can understand the relevant prior state in under 60 seconds
- Carers no longer maintain separate health diaries as the primary continuity mechanism

When these conditions hold, memory has moved into structure.

SECTION 13.1

WHAT IT LOOKS LIKE IN PRACTICE

Before the redesign

A patient is discharged from hospital after a short stay. A week later they attend a GP appointment. The discharge summary has not arrived. The GP asks the patient to explain what happened, what medication changed, what the hospital decided, and what follow-up was planned. The patient remembers some of it. They cannot recall the exact medication change. They did not bring the letter. The GP pieces together a clinical picture from fragments and makes a decision based on partial information.

After the redesign

The same patient attends the same GP appointment. Before the consultation begins, the GP opens the consultation packet. It shows the discharge event, the medication change, the pending follow-up, the current symptom the patient logged yesterday, and the last recorded clinical outcome. The GP reads it in under 60 seconds. The appointment begins with the case already assembled. The GP still judges. The GP still examines. The GP still decides. But they are starting from a complete picture rather than building one from scratch.

The GP's job does not change. The quality of the input to that job does.

SECTION 14

FAKE PROGRESS — HOW TO KNOW IF IT ISN'T WORKING

The redesign can be implemented in appearance while the old failure continues in structure. The following conditions confirm the primitive has returned, not that the redesign has succeeded:

App exists but patients still repeat their full history

The app is a health journal, not a continuity system. The packet is not changing the encounter.

Clinicians do not read the packet; appointments still begin with reconstruction

The 60-second rule has failed. The packet is a document, not a mechanism.

Success is measured by downloads and engagement rather than continuity retrieval

Growth metrics reward the audit layer. The operational test is retrieval before the clinical decision.

The record becomes institution-owned under NHS integration

The custody invariant is broken. The redesign has recreated the broken architecture in a new wrapper.

The app begins presenting symptom patterns as clinical diagnoses

The app has entered clinical authority it cannot hold. This is medical device territory.

Carers are still maintaining separate health diaries

The ghost structure has not dissolved. Burden has not moved from people into structure.

SECTION 15

RECAPTURE WARNING

The greatest risk after the redesign is built is recapture: the system quietly reinstalling the old failure under pressure.

Under pressure, the system will try to:

- Pull custody back into institutional systems — requiring the continuity record to be held by the NHS rather than the patient
- Convert continuity into audit documentation — optimising the record for compliance and legal defensibility rather than clinical retrieval
- Reintroduce reconstruction into the consultation — allowing the packet to become optional, ignored, or replaced by verbal history
- Measure the app by engagement metrics rather than continuity retrieval — turning it into a health journal

If any of these happen, the failure returns. The patient-held continuity object is not a preference. It is the corrected primitive. The redesign survives only if patient custody survives, retrieval remains decision-ready, and function separation is preserved.

Coercive access is also a recapture risk. Patient-held health records create risks that institution-held records do not: a family member, partner, carer, employer, or institution may pressure a patient to share sensitive health information. Patient-held custody only works if access is revocable at any time, sensitive information can be hidden from emergency display, and the patient can see a full history of who has accessed the record.

SECTION 16

HOW TO USE THIS DOCUMENT

AUDIENCE	USE THIS DOCUMENT TO...	THEN GO TO...
Leadership and policymakers	Understand the structural failure, the cost of continuation, and the direction of the replacement	Redesign Executive Summary (Artifact 02a) for the replacement logic and dependency order
Clinical teams	See what burden must leave the consultation and what the redesigned encounter looks like in practice	Diagnostic Artifact (Artifact 01) for the full evidence base
Patients and carers	Understand why history repetition is structural, not personal, and what the redesigned route changes for lived experience. For direct patient-facing use, produce a one-page summary drawn from Sections 3, 8, 12, and 13.	Translation Artifact (this document) is the starting point; share Sections 3 and 8 directly with patients
Builders and technical teams	Use the move order as orientation and the function allocation as context	Redesign Artifact (Artifact 02) for the full technical specification, schema, invariants, and testability requirements

SECTION 17

ONE-PAGE LEADERSHIP SUMMARY

Five questions a board member needs answered

LEADERSHIP QUESTION	ANSWER
What is failing?	Operational memory is absent at the point of care. The patient has become the record. The GP consultation collapses four system functions into one human node under time compression.
Why have previous fixes not worked?	They improved records and access — not decision-ready retrieval. The NHS has built extensive audit trace; it has not built operational memory. Every new digital layer has relocated the failure rather than corrected it.
What must replace it?	A patient-held operational continuity record, with a consultation packet readable in under 60 seconds at every encounter. The patient holds custody. The system carries memory and preparation. The clinician carries authority.
What must not happen?	Institutional custody transfer; diagnosis by app; engagement-metric optimisation; PDF fallback becoming the product; NHS integration attempted before Moves 1–6 are complete.
What proves it worked?	Same pressure → different response: no routine history repetition, GP reads packet in under 60 seconds, outcome appended after each encounter, carer's health diary no longer structurally necessary.

Detailed reference

ITEM	SUMMARY
Governing diagnosis	Structural memory is absent at the point of care; the GP consultation collapses four system functions into one human node under consequence.
Hidden burden (ghost structure)	Patients carry history; carers carry continuity; GPs reconstruct; receptionists route informally. These are not support roles — they are the missing system.
Cost of continuation	£2.8bn annual clinical negligence; 36 coroner PFD warnings linked to information-sharing failures; GP exit pressure; diagnostic delay; rising clinical risk. (Sources: Diagnostic Artifact.)
Event vs state mismatch	The system is optimised for events (appointments, referrals, discharges). The patient is state-based: a continuous clinical condition that exists between appointments. The system does not remember between events unless a human carries the memory.
Corrected primitive	Operational continuity must be present at the point of clinical consequence, in patient-held form. Clinical authority remains with the clinician.
Redesigned object	Patient-held operational continuity record: history, symptom events, consultation packet (readable in under 60 seconds), outcome append, emergency access, carer access.
The allocation	The patient holds custody. The system carries memory and preparation. The clinician carries authority.
Move order	Truth kernel → symptom layer → consultation packet → outcome append → emergency access → carer access → NHS recognition (Move 7 last, not first).
What must stop	Patient as memory, GP reconstruction as normal work, documentation as continuity, access metrics as success, routing prompts as diagnosis.
Proof of success	GP reads consultation packet in under 60 seconds; patient no longer repeats history; system functions when patient cannot speak; carer's health diary no longer structurally necessary.
Recapture risk	Institutional custody transfer, audit conversion, engagement-metric optimisation, or coercive access reinstalls the failure. Patient custody and 60-

ITEM	SUMMARY
	second retrieval must be preserved.

CLOSING STATEMENT

The diagnosis is not that NHS primary care lacks effort, records, or digital programmes. The diagnosis is that operational memory is absent where clinical decisions are made. The redesign is not another record system. It is a function reallocation: the patient holds custody, the system carries memory and preparation, and the clinician carries authority. For evidence, use the Diagnostic Artifact. For implementation, use the Redesign Artifact. For shared understanding, use this document.

This Translation Artifact is derived from the full Diagnostic and Redesign Artifacts. It introduces no new claims. For complete evidence, dependency ordering, and implementation specification, refer to those documents.

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